BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed MRI and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

Check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver	FBC, renal/liver profile, glucose; baseline lung function	FBC, renal and liver tests before each treatment	Lomustine SmPC: blood count at least weekly for at least 6 weeks after each dose	Review the lowest counts and recovery before another cycle	Follow persistent blood, lung, kidney or liver effects
Lungs / bowel / peripheral nerves	Baseline lung function; record bowel and nerve symptoms	Review before treatment; repeat lung testing with prolonged therapy or symptoms	Call for cough, breathlessness, severe constipation or new weakness	Check whether each PCV component remains suitable	Continue review of unresolved effects
Neurology / medicines / function	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00658 v2 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

PCV chemotherapy · 56-day cycle: treatment and checks

NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy.

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 56 days. Six cycles are planned, with review before treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00658	FBC, renal/liver profile, glucose; baseline lung function	FBC, renal and liver tests before each treatment	Lomustine SmPC: blood count at least weekly for at least 6 weeks after each dose	Review the lowest counts and recovery before another cycle	Follow persistent blood, lung, kidney or liver effects
Lungs / bowel / peripheral nerves NCCP 00658	Baseline lung function; record bowel and nerve symptoms	Review before treatment; repeat lung testing with prolonged therapy or symptoms	Call for cough, breathlessness, severe constipation or new weakness	Check whether each PCV component remains suitable	Continue review of unresolved effects
Neurology / medicines / function NCCP 00658	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00658	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1 PCV · 56-day cycle

Three medicines on different days

This eight-week PCV regimen combines procarbazine, lomustine and vincristine after radiotherapy for grade II glioma. It aims to improve disease control. NCCP describes this indication as unlicensed; the treating team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00658

2 Recovery and the next-cycle decision

Delayed blood-count effects

This eight-week PCV regimen combines procarbazine, lomustine and vincristine after radiotherapy for grade II glioma. It aims to improve disease control. NCCP describes this indication as unlicensed; the treating team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00658

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38